

ETHIOPIAN HEALTH PROFESSIONALS LICENSING EXAMINATION(EHPLE) PRACTICE TEST BOOKLET 2025

EMERGENCY & CRITICAL CARE NURSING



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION!



Message from the State Minister, Ministry of Health – Ethiopia



Mrs. Frehiwot Abebe

Improving the quality of healthcare remains a critical global priority and is essential for achieving sustainable development and universal health coverage. One of the most effective ways to ensure high standards in healthcare delivery is through the consistent assessment of professional competencies. In line with this commitment, the Ministry of Health of Ethiopia launched the Ethiopian Health Professionals Licensing Examination (EHPLE) in 2019 for undergraduates in seven core health disciplines. Since its inception, the program has expanded to encompass 17 health professions, reflecting our continued effort to build a competent and responsive health workforce.

The EHPLE was established with the primary aim of identifying and certifying health professionals who possess the minimum competencies required to provide safe and effective care. This ensures that all practicing professionals are equipped to meet the expectations of the communities they serve. The exam is coordinated by the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), which consists of four specialized desks. The LEO plays a vital role in system strengthening, enabling the exam to be conducted with increased efficiency, organization, and quality assurance.

It is important to recognize that the EHPLE is not a conventional academic or recruitment examination. Rather, it is a rigorous, competency-based assessment designed to evaluate the practical skills and critical thinking abilities that are essential for professional practice. This *Practice Items Booklet* is part of our broader strategy to support examinees and educators in preparing for the licensing process. It offers a realistic overview of the exam structure, promotes a deeper understanding of its objectives, and fosters transparency and sustainability in implementation.

The development of this booklet has been made possible through the collaborative efforts of experts from higher education institutions, the Addis Ababa University Testing Center, and professional associations. Their insights and dedication have been instrumental in shaping this resource. I extend my heartfelt thanks to all contributors and to the Ministry of Health team for their tireless work and steadfast commitment to excellence in healthcare regulation.

We believe this booklet will serve as a valuable tool for candidates preparing for the examination and for educators guiding them toward success. Together, we are building a stronger, more competent health workforce for Ethiopia.

Introduction EHPLE

Welcome to the 2025 Edition of the Ethiopian Health Professionals Licensing Examination Practice Test Booklet.

This resource is designed to support candidates in their preparation for the national licensing examination, ensuring they are well-equipped with the knowledge and clinical reasoning skills required to deliver competent and ethical healthcare across Ethiopia.

In Ethiopia, the licensing examination serves as a pivotal tool in evaluating the competency of health professionals who have completed their academic training and are ready to transition into independent practice. The exam not only ensures public safety by verifying professional readiness but also upholds national and international standards in healthcare service delivery. As the healthcare system continues to expand and evolve, a strong, well-regulated professional workforce becomes even more essential.

This practice booklet contains 200 multiple-choice questions modeled after the structure, content, and format of the actual licensing examination. The questions span across core clinical domains essential for entry-level practice. This includes areas such as Patient care, leadership and management, ethics, and Health Promotion as outlined in the Information Booklet 2024.

The booklet is intended to:

- Familiarize candidates with the structure and cognitive level of the actual licensing examination.
- Offer opportunities for self-assessment and identification of strengths and gaps in knowledge.
- Encourage critical thinking and clinical decision-making in scenarios reflective of real-world healthcare practice in Ethiopia.
- Reduce exam-related anxiety by providing realistic practice experiences.

Administered under the auspices of the Ministry of Health and relevant regulatory bodies, the Ethiopian Health Professionals Licensing Examination is grounded in evidence-based frameworks and aligned with both national needs and global best practices. It has been structured to assess a range of competencies including medical knowledge, communication, professionalism, and safe clinical practice.

This practice test is not a predictive tool, and performance on it does not guarantee similar outcomes on the actual exam. However, it provides an invaluable opportunity for familiarization with question styles, pacing strategies, and content review. We strongly recommend that candidates simulate real testing conditions while using this resource, including timing their responses and reviewing rationale for both correct and incorrect answers.

The goal of licensing examinations is not only to regulate entry into the profession but also to inspire excellence and lifelong learning. By engaging with this practice test, you are taking a proactive step toward professional development, accountability, and a commitment to serving your patients with competence and compassion.

We wish you success on your licensing journey—and beyond.

Acknowledgements

This *Practice Test Items Booklet for the Ethiopian Health Professionals Licensing Examination* is the result of a collaborative effort among dedicated professionals committed to advancing the quality of health sciences education in Ethiopia. The Ministry of Health extends its sincere appreciation to all those who contributed their time, expertise, and insights to the development of this essential resource.

Special recognition is given to the item developers and reviewers from various higher education institutions—both public and private—whose academic rigor and practical experience were instrumental in shaping the content and ensuring its relevance to real-world healthcare practice. Their contributions have helped create a high-quality, standardized practice tool that reflects national competencies and global benchmarks.

We are especially grateful to the team at the Addis Ababa University Testing Center, whose technical support and academic guidance were critical throughout the process. Their leadership in assessment design and educational evaluation has significantly strengthened the quality of this booklet.

We also thank the committed staff of the Ministry of Health, particularly those within the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (HHRIPR LEOs), whose coordination, oversight, and unwavering dedication ensured the successful completion of this project.

Together, these collective efforts reflect our shared vision of building a competent, ethical, and resilient health workforce for Ethiopia.

Emergency and Critical Care Nursing

1. A 35-year-old female patient came to an emergency department with complaints of chest pain, cough, and fever of six days duration. On physical examination, the respiratory rate of 15 breaths/minute, heart rate of 110 beats/minute, blood pressure of 100/70mmHg, and temperature of 41 degrees Celsius were recorded. *She* is awake and able to communicate clearly.

What is the most appropriate triage category for this patient?

- A. Green
- B. Yellow
- C. Orange
- D. Red

ANSWER: C

2. A 20-year-old female patient came to an emergency department with difficulty breathing, productive cough, and chest tightness. On assessment, *she* has a wheezing sound over the chest, and fatigue while walking. Respiratory rate of 24 breaths/minute; heart rate 111 beats/minute, and temperature of 36.5 degrees centigrade were noticed.

What is the most likely diagnosis in this case?

- A. Pleural effusion
- B. Acute asthma
- C. Pneumonia
- D. Respiratory failure

ANSWER: B

3. A 55-year-old female patient came to an emergency department with complaints of chest pain, productive cough with frothy sputum, labored breathing, and difficulty breathing during sleep. On assessment, the respiratory rate of 24 breaths/minute, the pulse rate of 120 beats/minute, and blood pressure of 110/70mmHg are recorded. Previously, *she* had an admission history and was treated for pulmonary fluid edema.

What is the most relevant history requested by the nurse from the patient?

- A. Past Medical history
- B. Sputum character
- C. Work history
- D. Vital signs

ANSWER: B

4. A 25-year-old female patient came to an emergency department with complaints of difficulty breathing, cough, and chest tightness of 10 days duration. The cough was characterized by white frothy sputum. Initial assessment showed respiratory rate of 22 breaths/minute, heart rate of 120 beats/minute, and blood pressure of 110/80mmHg, PaO₂ of 70mmHg, PaCO₂ of 55mmHg, HCO₃ of 24mEq/L and PH of 7.30.

What is the most likely diagnosis in this case?

- A. Respiratory Alkalosis
- B. Metabolic alkalosis
- C. Respiratory acidosis
- D. Metabolic acidosis

ANSWER: C

5. A 50-year-old patient came to emergency department with complaints of shortness of breath and chest tightness. The patient has a history of smoking. He takes an Albuterol puff daily, but has poor compliance with the medication. On assessment, he uses accessory muscles to breathe with a respiratory rate of 24 breaths/minute and loud wheezes heard on the right side.

What is the most appropriate patient history that describes this condition?

- A. Accessory muscle use
- B. Continued smoking
- C. Inability to supine lie down
- D. Altered level of consciousness

ANSWER: B

6. A 75-year-old male patient came to an emergency department with complaints of shortness of breath and mild chest pain. He had a history of smoking and cardiac disease. On physical examination, he has pale skin and a contracted heart rate of 130 beats/minute, a respiratory rate of 22 breaths/minute, blood pressure of 90/70mmHg, and a temperature of 38 degrees Celsius.

What is the appropriate actual nursing diagnosis of this patient?

- A. Imbalanced nutrition is less than the body's requirement
- B. Impaired gas exchange related to mucosal contracture
- C. Knowledge deficit about health risk prevention measures
- D. Risk for airway compromise and chronic disease

ANSWER: B

7. A 55-year-old female patient came to an emergency department with complaints of shortness of breath, headache, and anxiety for the past three days. Two days before, she started treatment for pleural effusion. On assessment, her chest is clear, uses accessory muscles with a respiratory rate of 27 breaths/minute, a heart rate of 140 beats/minute, and blood pressure of 90/60mmHg.

What other physical finding most likely describes the condition of this patient?

- A. Diminished breath sound
- B. Crackles and diaphoresis
- C. Asymmetry of the chest
- D. Hemoptysis

ANSWER: A

8. A 40-year-old male patient came to an emergency department with complaints of shortness of breath and a non-productive cough. On physical examination, a wheezing sound is heard over his chest, but he has no crackles or breath sounds. Despite oxygen administration, his saturation dropped to 75 percent. His vital signs indicated a respiratory rate of 40 breaths/minute and a pulse rate of 120 beats/minute. Arterial blood gases showed type I respiratory failure.

What is the most likely diagnosis for this patient?

- A. Pulmonary edema
- B. COPD exacerbation
- C. Severe pneumonia
- D. Plural effusion

ANSWER: A

9. A 40-year-old known rheumatic disease male patient came to an emergency department with an acute exacerbation of chronic obstructive pulmonary disease and was admitted to a hospital for better management. On physical examination, his chest looks barrel-shaped and has paradoxical chest movement. Respiratory and pulse rates are 20 breaths/minute and 130 beats/minute, respectively. On auscultation, he has an inspiratory crackling sound.

What is the most relevant cause for admitting this patient?

- A. Insufficient support
- B. Co-morbidities infection
- C. Shortness of breath
- D. Abnormal chest shape

ANSWER: B

10. A 30-year-old male patient came to an emergency department with complaints of shortness of breath, fever, chills, fatigue, and loss of appetite. *She* had a history of admission to a COVID rehabilitation center for the past month. On physical examination, oxygen saturation of 75 percent, respiratory rate of 27 breaths/minute, heart rate of 130 beats/minute, and blood pressure of 125/85mmHg were noticed. He was diagnosed with respiratory distress syndrome.

What is the most likely cause of the patient's problem?

- A. Pneumonia
- B. Bronchial asthma
- C. Pneumothorax
- D. COPD exacerbation

ANSWER: A

11. A 28-year-old male patient was admitted to an adult intensive care unit with severe pneumonia. On admission, he had shortness of breath, chills, and fever. An endotracheal tube is inserted for airway management. On physical examination, a respiratory rate of 26 breaths/minute, a heart rate of 110 beats/minute, and a temperature of 39 degrees Celsius and oxygen saturation of 65 percent were noticed.

What is the most appropriate initial nursing care plan for this patient?

- A. Daily patient ambulation
- B. Weekly oral care with saline
- C. Deep venous thrombosis prophylaxis
- D. Daily sedation of the patient

ANSWER: D

12. A 30-year-old female patient came to an emergency department with complaints of sore throat, fever, headache, and sweating for 15 15-day duration. *She* has been living with HIV for about 10 years. Previously, *she* was hospitalized for respiratory opportunistic infections. Physical examination showed fine crackle, respiratory rate of 25 breaths/minute, pulse rate of 120 beats/minute, temperature of 39 degrees Celsius, and blood pressure of 120/70mmHg.

What is the most likely cause of the patient's problem?

- A. H. influenza
- B. Pneumocystis jirovecii
- C. Mycobacterium tuberculosis
- D. Mycobacterium ovium

ANSWER: B

13. A 45-year-old male patient came to an emergency department with shortness of breath, chest pain, and a chronic cough. He had a history of smoking and drinking alcohol. On examination, he had an acute exacerbation of COPD.

What is the most likely persistent manifestation in this case?

- A. Productive cough and chest tightness
- B. Wheezing and chest pain
- C. Chronic Cough and dyspnea
- D. History of smoking and drinking

ANSWER: C

14. A 35-year-old male patient came to an emergency department with complaints of cough, fever, and shortness of breathing. Physical examination showed crackles on breathing with a respiration rate of 25 breaths/minute, a heart rate of 110 beats/minute, and a temperature of 38 degrees Celsius.

What is the most appropriate nursing management for this patient?

- A. Initiate mechanical ventilation
- B. Initiate physiotherapy
- C. Provide a high flow of oxygen with a face mask
- D. Providing a high flow of oxygen with a nasal catheter

ANSWER: C

15. A 25-year-old male patient came to an emergency department with complaints of stabbing chest pain, fever, headache, and fatigue for 7 7-day duration. He rarely smokes cigarettes and drinks alcohol. A month before, he had been hospitalized due to HIV/AIDS associated pneumonia. On physical examination, a respiratory rate of 22 breaths/minute, a heart rate of 110 beats/minute, and a blood pressure of 110/80mmHg were noticed.

What is the most appropriate preventive measure for this condition?

- A. Encourage smoking cessation
- B. Promote lung expansion exercise
- C. Initiate special precautions against infection
- D. Encourage reduced and moderate alcohol intake

ANSWER: C

16. A 28-year-old patient came to an emergency department because of shortness of breath, severe chest pain, and loss of appetite. He has a family history of lung cancer and heart disease. Physical examination showed a respiratory rate of 30 breaths/minute, a pulse rate of 110 beats/minute, blood pressure of 120/90mmHg. The CT scan reading also showed fluid in the thoracic cavity.

What is the most priority nursing management of this patient?

- A. Prepare and position the patient for thoracentesis
- B. Administer analgesic as prescribed and as needed
- C. Prepare the patient for chest tube insertion and water seal drainage
- D. Prepare the patient for open tube thoracostomy for chest drainage

ANSWER: B

17. A 40-year-old male patient who was newly diagnosed with asthma came to an emergency department with complaints of difficulty breathing, chest tightness, and productive cough. In history, he is working in a leather factory and he had never smoked a cigarette. On physical examination, loud wheezing, respiratory rate of 25 breaths/minute, heart rate of 110 beats/minute, and blood pressure of 120/80mmHg were recorded.

What is the most appropriate preventive method for the patient's condition?

- A. Evaluation of impairment and risk factors
- B. Administration of short-acting bronchodilators
- C. Identification of precipitating factor
- D. Evaluation and identification of work-related history

ANSWER: C

18. A 30-year-old male patient came to an emergency department with chest pain for two days. Physical examination showed decreased chest breathing sound, asymmetrical chest, respiratory rate of 20 breaths/minute, pulse rate of 120 beats/minute, and oxygen saturation of 90 percent. Ultrasound result showed air in the thoracic cavity.

What is the appropriate nursing management in this case?

- A. Place the patient in a semi-upright position
- B. Use sterile gauze and apply a side dressing
- C. Place the patient supine and provide supplemental oxygen
- D. Prepare the patient for intubation and better management

ANSWER: A

19. A 45-year-old male patient came to an emergency department after surviving of crash injury with complaints of chest pain and difficulty breathing. On physical examination, bruising on the anterior chest, use of accessory muscles, respiratory rate of 27 breaths/minute, and pulse rate of 130 beats/minute were noticed.

What is the most likely priority nursing management in this case?

- A. Provide wound care and prevent infection
- B. Pain management and adequate ventilation
- C. Maintain adequate fluid intake
- D. Maintain adequate nutritional status

ANSWER: B

20. A 70-year-old male patient came to an emergency department with complaints of chest pain and a productive cough. On physical examination, he has purulent sputum, oxygen saturation of 89 percent, respiratory rate of 20 breaths/minute, heart rate of 120 beats/minute, and blood pressure of 110/90mmHg. He is diagnosed with chronic obstructive pulmonary disease.

What is the most appropriate nursing management in this case?

- A. Chest physiotherapy with percussion and postural drainage
- B. Use of the huff cough technique with a specified breathing pattern
- C. Oxygen therapy and decreased fluid intake as needed
- D. Internal and parenteral nutrition may require

ANSWER: A

21. A 47-year-old male patient with lung cancer came to an emergency room with complaints of difficulty breathing at rest and occasional sharp chest pain that worsens on inhalation. Dullness on percussion and decreased breath sounds are noted in the lower lung fields. The patient's respiratory rate is 27 breaths/minute, pulse rate is 120 beats/minute, blood pressure is 95/60mmHg, and temperature is 36.5 degrees centigrade. An upright chest radiograph shows blunting of the costophrenic angle.

What is the most appropriate initial management for this patient?

- A. Thoracentesis
- B. Paracentesis
- C. Postural drainage
- D. Laparotomy

ANSWER: A

22. A 26-year-old male patient was admitted to an emergency department following trauma, and he was transfused with whole blood. After 12 hours, the patient started complaining of difficulty breathing, became agitated, and restless. Vital signs showed a respiratory rate of 29 breaths/minute; pulse rate of 105 beats/minute, and temperature of 38 degrees centigrade. The chest X-ray indicates patchy bilateral infiltrates.

What is the most appropriate initial intervention for this patient?

- A. Oxygenate with the highest fraction of inspired oxygen
- B. Oxygenate with low tidal volumes
- C. Oxygenate with lower peak end expiratory pressure
- D. Elevate plateau pressures to higher than 30 cm H₂O

ANSWER: B

23. A 56-year-old patient came to an emergency room with a complaint of difficulty breathing at rest for 3 3-day duration. In addition, the patient reports occasional sharp, non-radiating chest pain that worsens on inhalation. On physical examination, percussion dullness and decreased breath sounds are noted in the dependent areas of the lung. The patient's respiratory rate is 24 breaths/minute, pulse rate is 98 beats/minute, blood pressure is 115/70mmHg, and temperature is 36.5 degrees centigrade.

What is the most appropriate priority nursing intervention in this case?

- (A) Prepare the patient for postural drainage
- (B) Prepare the patient for thoracentesis
- (C) Prepare the patient paracentesis
- (D) Prepare the patient for bronchoscopy

ANSWER: B

24. A 47-year-old male patient came to an emergency department with complaints of difficulty breathing, cough, and fatigue. His respiratory rate is 29 breaths/minute, pulse rate is 115 beats/minute, blood pressure of 95/60mmHg, and oxygen saturation is 85 percent.

What is the priority nursing intervention for this patient?

- A. Administer oxygen
- B. Administer Antibiotics
- C. Administer analgesics
- D. Administer antipyretics

ANSWER: A

25. A 60-year-old male patient on sepsis treatment started complaining of difficulty breathing. Associated with this, the patient is restless, agitated, and uses respiratory accessory muscles. Vital signs showed a respiratory rate of 29 breaths/minute, of pulse rate of 105 beats/minute, and a temperature of 38 centigrade. The chest X-ray indicates patchy bilateral infiltrates. The physician suspected acute respiratory distress syndrome. The patient is intubated and put on mechanical ventilation with a tidal volume of 10mL/kg of predicted weight and high fraction of inspired oxygen (FiO₂).
For what complication is this patient at increased risk?
- A. Pneumothorax
 - B. Pulmonary embolism
 - C. Pleural effusion
 - D. Pulmonary hypertension
- ANSWER: A
26. A 26-year-old male patient with sustained blunt injury to the chest presented to an emergency department with a complaint of difficulty breathing of two hours duration. Hemopneumothorax was diagnosed, and water-sealed chest tube drainage was inserted. The symptom was relieved after 12 hours. The nurse in charge clamped the chest tube to replace the chest tube drainage unit that was full and forgot to remove the clamp for a while.
For what complication is this patient at greater risk?
- A. Pulmonary embolism
 - B. Pulmonary hypertension
 - C. Tension pneumothorax
 - D. Pulmonary Effusion
- ANSWER: C
27. A 59-year-old female patient is on mechanical ventilation because of acute respiratory failure. The nurse did not check the cuff pressure of the endotracheal tube for the last 12 hours. The endotracheal tube is suddenly extubated while a critical care nurse is positioning the patient. Then, the patient is re-intubated.
For what complications is the patient at increased risk?
- A. Gastritis/heartburn
 - B. Aspiration into the lung
 - C. Esophageal intubation
 - D. Vomiting/hemoemesis
- ANSWER: B
28. A 43-year-old known asthmatic female patient came to the hospital for follow-up. She has been receiving systemic corticosteroids to control inadequately treated persistent asthma.
For what complication is the patient at increased risk?
- A. Fluid retention
 - B. Hypotension
 - C. Dehydration
 - D. Pulmonary edema
- ANSWER: A

29. A 61-year-old male patient with acute respiratory failure is admitted to an intensive care unit. He is intubated and on mechanical ventilation. He tries to express his feelings to the attending nurse using different facial expressions and body language.
For what complication is this patient at increased risk?
A. Impaired mobility
B. Impaired communication
C. Impaired nutrition
D. Impaired gas exchange
ANSWER: B
30. A 48-year-old male patient with sustained chest trauma came to a busy emergency department. He is on treatment for traumatic pneumothorax. He is unconscious and positioned supine and flat on the bed.
For what complication is this patient at greatest risk?
A. Pulmonary embolism
B. Aspiration into the lung
C. Pericardial effusion
D. Core pulmonale
ANSWER: B
31. A 67-year-old known cardiac patient came to an emergency department due to shortness of breath. Physical examination reveals a heart rate of 125 beats per minute, blood pressure of 120/70mmHg, respiratory rate of 20 breaths per minute, and oxygen saturation of 91 percent on atmospheric air.
What is the most appropriate triage category for the patient?
A. Orange
B. Yellow
C. Green
D. Red
ANSWER: A
32. A 65-year-old male patient came to an emergency department with complaints of breathing difficulty and fatigue of one day duration. He states that the shortness of breath worsens when lying in the supine position and usually awakens him from sleep. On physical examination, an emergency nurse noted edema around the patient's ankles and then suspected the diagnosis of heart failure.
What is the most important additional history to be asked by the nurse to support the suspected diagnosis?
A. Dyspnea on exertion
B. History of hospitalization
C. Family history of heart failure
D. Fatigue that gets away after sleep
ANSWER: A
33. A 67-year-old male patient was admitted to an emergency department due to injury to the left upper part of his leg six hours ago. After twenty-four hours of his admission, the patient started to show signs and symptoms of thromboembolism.
What is the most likely risk factor for the occurrence of thromboembolism in this patient?
A. Age of the patient
B. Major bone fracture
C. Splinting procedure
D. Long emergency room stay
ANSWER: B

34. A 34-year-old male patient came to an emergency department with a complaint of chest pain for a day. He reports that he is feeling a sharp stabbing pain that squeezes his chest as if something heavy is sitting on it. The Nurse suspects acute coronary syndrome.
What is the most appropriate physical examination finding that supports the diagnosis?
- A. Nausea and vomiting
 - B. Confusion and lethargy
 - C. Hypotension and tachycardia
 - D. Absence of S3 heart sound and murmur
- ANSWER: C
35. A 72-year-old patient came to an emergency room with complaints of shortness of breath, foot swelling, and extreme tiredness. An emergency nurse suspects heart failure.
What is the most reliable diagnostic method to diagnose this case?
- A. Standard Echocardiography
 - B. Bedside ultrasonography
 - C. 12-lead Electrocardiography
 - D. Anteroposterior Chest X-ray
- ANSWER: A
36. A 41-year-old patient came to an emergency department with a third-degree burn. Vital signs show a blood pressure of 80/50mmHg, pulse rate of 125 beats per minute, respiratory rate of 27 breaths per minute, and oxygen saturation of 87 percent.
What is the most likely diagnosis in this case?
- A. Septic Shock
 - B. Neurogenic Shock
 - C. Hypovolemic Shock
 - D. Distributive shock
- ANSWER: C
37. A 56-year-old patient on treatment due to heart failure in an emergency department started to complain of chest pain. On cardiac monitoring, an emergency nurse noted that the alarming ECG shows a ventricular rate of 350 beats per minute without recognizable QRS complexes. Physical examination shows the absence of a central pulse.
What is the most likely diagnosis in this case?
- A. Atrial flutter
 - B. Atrial fibrillation
 - C. Ventricular fibrillation
 - D. Ventricular tachycardia
- ANSWER: C
38. A 35-year-old patient was admitted to an emergency room with hemolytic anemia. The patient reported easy fatigability and difficulty moving his hands and legs.
What is the most likely Nursing diagnosis in this case?
- A. Activity intolerance
 - B. Imbalanced nutrition
 - C. Ineffective tissue perfusion
 - D. Noncompliance with prescribed therapy
- ANSWER: A

39. A 70-year-old male patient with a history of smoking came to an emergency department with a complaint of chest discomfort of six hours duration. The discomfort is not relieved with rest, and he is anxious and restless. His electrocardiograph shows an ST-segment elevation at lead II. He is diagnosed with ST-segment Elevated Myocardial Infarction and was put on treatment.
What is the most likely priority nursing care plan in this case?
- A. To relieve pain or discomfort
 - B. To maintain respiratory function
 - C. To enhance gas exchange
 - D. To improve urine output
- ANSWER: A
40. A 38-year-old female patient came to an emergency department with complaints of difficulty breathing that worsens at night when lying in bed, and a cough of one week duration. Her condition is aggravated with exertion. *She* is diagnosed with acute heart failure, and *she* is on treatment.
What is the most appropriate priority nursing care plan in this case?
- A. Promoting activity and reducing fatigue
 - B. Relieving fluid deficit symptoms
 - C. Assisting a family member in managing the condition
 - D. Minimizing feelings of powerlessness
- ANSWER: A
41. A patient who was admitted to a hospital with acute coronary syndrome has continuous electrocardiography monitoring. An examination of the rhythm strip reveals a regular atrial rate of 74 beats/min, an irregular ventricular rate of 62 beats/min, a normal shape of P wave, PR interval lengthens progressively until a P wave is not conducted, and a normal shape of QRS.
What is the most appropriate priority nursing care plan in this case?
- A. Performing synchronized cardioversion
 - B. Administering epinephrine 1 mg intravenous push
 - C. Observing for symptoms of hypotension or angina
 - D. Applying transcutaneous pacemaker pads on the patient
- ANSWER: C
42. A 25-year-old female patient came to an emergency department with complaints of frequent nasal and gingival bleeding of two weeks duration. Complete blood count result shows a platelet count of 120,000/mm³. *She* is diagnosed with thrombocytopenia.
What is the most appropriate priority nursing care plan in this case?
- A. Encouraging to take aspirin
 - B. Teaching self-management
 - C. Encouraging to take heparin
 - D. Encouraging to take warfarin
- ANSWER: B
43. A patient was admitted to an intensive care unit with a diagnosis of heart failure. On hemodynamic monitoring, an emergency nurse detects signs of decreased cardiac output. The nurse is setting a treatment goal for the patient's intensive care stay.
What is the most important initial management goal in this case?
- A. To give symptomatic relief
 - B. To maintain adequate oxygenation of tissues
 - C. To ensure sinus rhythm on electrocardiography
 - D. To cure the patient from disease-associated symptoms
- ANSWER: B

44. A 25-year-old female patient was admitted to an emergency room with a diagnosis of septic shock. She has a history of unsafe abortion and foul smell of vaginal discharge. Vital signs show blood pressure of 90/75mmHg, Respiratory rate of 22 breaths per minute, and temperature of 38.5 degrees centigrade.

What is the most appropriate initial management in this case?

- A. Initiation of Broad-spectrum antibiotics
- B. Initiation of Cautious fluid resuscitation
- C. Performing surgical abscess drainage
- D. Beginning of blood transfusion

ANSWER: A

45. A 19-year-old patient came to an emergency room with a sudden loss of consciousness. At the time of arrival, the patient had no pulse and respiration. As a result, an emergency team immediately started cardiopulmonary resuscitation.

What additional management strategy is key for the patients' improvement and survival?

- A. Identifying and treating underlying causes
- B. Performing pacemaker implantation
- C. Fluid resuscitation with crystalloid
- D. Applying electrical cardioversion

ANSWER: A

46. A 47-year-old patient was admitted to an emergency department with a diagnosis of cardiogenic shock. The nurse put him on supplemental oxygen therapy.

What is the most appropriate initial fluid resuscitation strategy in this case?

- A. Rapid administration of 1000 ml of Normal Saline
- B. Cautious administration of 250 ml of Normal Saline
- C. Administration of 20ml/Kg fluid within 30 minutes
- D. Administration of 5ml/Kg fluid within 30 minutes

ANSWER: B

47. A patient was admitted to an intensive care unit with a diagnosis of disseminated intravascular coagulopathy (DIC). On examination, the nurse noted signs of hypotension, hypoxia, and acidosis.

What is the most definitive management strategy in this case?

- A. Blood transfusion
- B. Oxygen administration
- C. Administering vasodilators
- D. Treating underlying causes

ANSWER: D

48. A 56-year-old known hypertensive patient came to an emergency room with complaints of headache, visual disturbance, and confusion of one hour duration. Vital signs show blood pressure of 160/110mmHg, respiratory rate of 25 breaths per minute, and heart rate of 96 beats per minute. Laboratory investigation reveals excess urea in blood. Based on the assessment, an emergency team decided to reduce the blood pressure immediately.

What is the most appropriate initial drug choice in this case?

- A. Hydralazine
- B. Labetalol
- C. Nitroglycerin
- D. Sodium nitroprusside

ANSWER: D

49. A 48-year-old known hypertensive and diabetic patient was admitted to an emergency department with a diagnosis of acute cardiogenic shock five minutes ago. After further investigation, there was an infarction of the anterior cardiac wall.

What is the priority nursing intervention in this case?

- A. Initiate management to reduce infarct size
- B. Preparing the patient for cardioversion
- C. Preparing the patient for intubation
- D. Preparing the patient for ambulation

ANSWER: A

50. A 43-year-old male patient was kept in an emergency room with a diagnosis of Hypovolemic shock. The patient was kept on an intravenous fluid therapy.

What is the priority nursing intervention in this case?

- A. Monitoring blood pH
- B. Monitoring fluid balance
- C. Monitoring mental status
- D. Monitoring organ functions

ANSWER: B

51. A 46-year-old male patient was admitted to an emergency room after a head injury. Upon admission, physical examination revealed a GCS of 11/15. The physician ordered an MRI with contrast. After a while, the patient returned back immediately from the radiology unit because of Anaphylactic shock after administration of radio contrast dyes. On arrival to an emergency room, the patient developed cyanosis, stridor, and the GCS became 8/15.

What is the priority nursing intervention in this case?

- A. Administering blood products
- B. Monitoring organ function
- C. Preparing for intubation
- D. Monitoring GCS

ANSWER: C

52. A known pulmonary Tuberculosis patient came to an emergency department with a history of progressive sharp chest pain of four hours duration. The pain worsens during deep inhalations and is aggravated when the patient lies in the supine position. The patient was diagnosed with acute pericarditis.

What is the priority nursing intervention in this case?

- A. Management of organ injury
- B. Management of infection
- C. Monitoring intake and output
- D. Management of pain and anxiety

ANSWER: D

53. A 46-year-old female patient was referred to a hospital due to chronic fatigue, spontaneous bleeding, and was unable to stop bleeding even after a minor cut. On investigation, the platelet count was 110,000 per microliter ($150-400 \times 10^3$ per microliter), indicating Thrombocytopenia.

What is the priority nursing intervention in this case?

- A. Health education
- B. Monitoring vital signs
- C. Prevention of bleeding
- D. Administration of medications

ANSWER: C

54. A 20-year-old female patient came to a hospital with a history of nausea, vomiting, and diarrhea of six hours duration. On arrival, her blood pressure is 80/60mmHg; pulse rate is 95 beats per minute; respiratory rate is 16 breaths per minute; temperature is 37.9 degrees centigrade; and oxygen saturation is 98 percent.
What is the most appropriate initial nursing intervention in this scenario?
- A. Placing the patient in the Trendelenburg position
 - B. Administering broad-spectrum antibiotics
 - C. Monitoring organ function
 - D. Administering anti-pain
- ANSWER: A
55. A patient was admitted to an intensive care unit with a diagnosis of myocardial infarction. The patient starts to complain of chest pain. Physical examination shows rapid and thready pulse, narrow pulse pressure, and distended neck veins.
What is the most likely cardiac complication in this case?
- A. Cardiogenic shock
 - B. Hypotension
 - C. Pericarditis
 - D. Septic shock
- ANSWER: A
56. A 55-year-old female patient was admitted to an emergency department with a diagnosis of severe anemia. The patient has not improved despite multiple interventions for Anemia.
What is the most likely immediate complication in this case?
- A. Heart Failure
 - B. Hemorrhagic Stroke
 - C. Pulmonary embolism
 - D. Valvular Heart Disease
- ANSWER: A
57. A 47-year-old male patient was admitted to an intensive care unit with bleeding from the surgical site after his post-operative recovery. The Nurse puts the patient on 100 percent oxygen and administers 1000ml of normal saline, but the patient's oxygen saturation does not show improvement.
What is the most likely complication that will occur in this scenario?
- A. Thrombus formation
 - B. Polycythemia Vera
 - C. Coagulopathy
 - D. Anemia
- ANSWER: D
58. A patient was admitted to an intensive care unit with a diagnosis of myocardial infarction. The patient is receiving oxygen, Aspirin, and Nitroglycerine.
What is the most important factor that has to be monitored in this case?
- A. Occurrence of cough
 - B. Signs of internal bleeding
 - C. Development of sinus rhythm
 - D. Effectiveness of pain medications
- ANSWER: B

59. A 55-year-old male patient came to an emergency department with a penetrating head injury. On examination, he was unconscious and vital signs showed blood pressure of 70/50mmHg, pulse rate of 55 beats per minute, respiratory rate of 8 breaths per minute, temperature of 37.9 degrees centigrade, GCS 7/15, and oxygen saturation of 88 percent.

What most appropriate triage category that signifies this scenario?

- A. Red
- B. Yellow
- C. Green
- D. Black

ANSWER: A

60. A 38-year-old patient came to an emergency room with complaints of nausea, vomiting, abdominal pain, constipation, and muscle paralysis after 24 hours of eating processed and packed food. The emergency nurse suspected that the diagnosis was botulism.

What is the most likely expected characteristic of paralysis in the history of this patient?

- A. Descending and symmetric paralysis
- B. Descending and asymmetric paralysis
- C. Ascending and asymmetric paralysis
- D. Ascending and symmetric paralysis

ANSWER: A

61. A patient with ischemic stroke was admitted to an intensive care unit. On assessment, the patient had a history of deep vein thrombosis and had not received any treatment before admission.

What is the most appropriate investigation in this scenario?

- A. Brain MRI
- B. Platelet count
- C. Prothrombin time
- D. Cerebral angiography

ANSWER: D

62. A 28-year-old female patient came to an emergency room with muscle weakness that progressed upward, and paresthesia of the lower extremities. On assessment, motor reflex was diminished, and bilateral body weakness was noted. Cognitive and consciousness levels were intact.

What is the most likely diagnosis in this case?

- A. Guillain-Barré syndrome
- B. Acute meningitis
- C. Myasthenia gravis
- D. Multiple sclerosis

ANSWER: A

63. A 50-year-old male patient with a suspected diagnosis of Guillain-Barre syndrome referred from a health center presented to an emergency room with complaints of ascending muscle weakness, quadriplegia, and difficulty breathing for a 1-hour duration. Vital signs revealed blood pressure of 100/70mmHg, respiratory rate of 30 breaths per minute, and pulse rate of 80 beats per minute.

What is the most likely actual nursing diagnosis in this case?

- A. Imbalanced nutrition
- B. Impaired verbal communication
- C. Ineffective breathing pattern
- D. Risk for fear and anxiety related to disease

ANSWER: C

64. A patient with Myasthenia gravis came to an emergency department with a myasthenic crisis. After two days of treatment, the patients' health status improved, and planned to be discharged. An emergency nurse is preparing the patient for discharge.
What most appropriate nursing care plan should the nurse to counsel on?
- A. Providing curative options for the disease
 - B. Patient and family education
 - C. Disease prevention
 - D. Physical exercise
- ANSWER: B
65. A patient with altered mental status came to an emergency room. An emergency team is investigating the cause of the mental status change. Vital signs show a respiratory rate of 20 breaths per minute, blood pressure of 110/80mmHg, and pulse rate of 80 beats per minute. An emergency nurse is planning to provide nursing care until the final diagnosis is confirmed.
What is the most appropriate nursing care plan in this case?
- A. Implementing safety measures
 - B. Providing respiratory support
 - C. Providing health education
 - D. Ensuring tissue perfusion
- ANSWER: A
66. A 36-year-old male patient came to an emergency room with a history of ingestion of contaminated packaged food. On assessment, the patient had nausea, vomiting, diarrhea, and difficulty swallowing. On physical findings, an emergency nurse noted a dry mouth and suspected the diagnosis of Botulism. What is the most appropriate nursing care plan in this scenario?
- A. Monitoring hydration and nutritional status
 - B. Monitoring serum toxin level
 - C. Monitoring organ functions
 - D. Monitoring mental status
- ANSWER: A
67. A 38-year-old known hypertensive female patient was presented to the adult emergency unit. On physical examination, the patient is restless, confused, has sluggish pupillary reactions, increased systolic blood pressure, and bradycardia. Then, the patient was diagnosed with increased intracranial pressure.
What is the most appropriate management in this case?
- A. Normal saline 0.9 percent
 - B. Dextrose 5 percent in saline
 - C. Mannitol IV infusion
 - D. Furosemide IV infusion
- ANSWER: C

68. A 60-year-old male patient came to a hospital emergency unit with the sudden onset of right-side facial droop, numbness, visual change, and aphasia. Physical examination shows blood pressure of 160/105, heart rate of 70 beats per minute, respiration of 22 breaths per minute, temperature of 37.8 degrees Celsius, and blood glucose level of 108 mg/dl. The patient is well oriented, but has difficulty speaking and sluggish pupillary reactions.

What is the primary focus of initial management for this patient?

- A. Control of blood pressure level
- B. Control of blood glucose level
- C. Restoration of cerebral blood flow
- D. Prevention of mechanical clot

ANSWER: C

69. A 30-year-old and, seven months pregnant woman came to an emergency unit shortly after an episode of seizure. *She* has no history of seizure, epilepsy, or any other chronic disease, and does not remember any trauma. On physical examination, *she* has a heart rate of 88 beats per minute, respiration of 18 breaths per minute, temperature of 36.5 degrees Celsius, and blood pressure of 158/98mm/Hg, and a random blood glucose level of 80 mg/dl.

What is the most likely management for this patient?

- A. Propofol IV infusion
- B. Diazepam IV infusion
- C. Fosphenytoin IV infusion
- D. Magnesium sulfate IV infusion

ANSWER: D

70. A 42-year-old male driver presented to the emergency department with the sudden onset of left-sided body weakness, numbness, and aphasia of six hours duration. On physical examination, the patient has sluggish pupillary reactions, blood pressure of 186/120 mmHg, a heart beats of 60 beats per minute, 32 breaths per minute, and altered mental status.

What is the most appropriate initial management for this patient?

- A. Administer oxygen with a face mask
- B. Establish airway and administer oxygen
- C. Save the ischemic area as much as possible
- D. Reduce blood glucose level to the normal range

ANSWER: B

71. A 33-year-old female patient came to an emergency department with decreased muscle strength, weakness of proximal muscles, and fatigue that worsens after repetitive use of voluntary muscles. The patient has recently been treated for herpes simplex virus. On physical examination, the patient has generalized body weakness that is most pronounced in the extremity muscle groups, neck extensors, facial, and bulbar muscles. *She* has ptosis, diplopia, dysphagia, and dyspnea. The patient is diagnosed with myasthenia gravis.

What is the most appropriate emergency nursing intervention in this case?

- A. Administering IV immunoglobulin.
- B. Establish airway with intubation
- C. Maintain adequate body fluid volume
- D. Administering oxygen with a face mask

ANSWER: B

72. A 44-year-old male patient presented to an emergency department with a severe headache, fever, and neck stiffness of two days duration, accompanied by seizure activity. The patient has reported chronic sinusitis and has no history of seizure disorders and trauma. Physical examination reveals 118 heart beats per minute, 18 breaths per minute, a temperature of 38.8 degrees Celsius, and blood pressure of 138/88 mm/Hg, and a random blood glucose level of 130 mg/dl.

What is the most appropriate priority nursing intervention in this case?

- A. Administer IV corticosteroids
- B. Administer IV antihypertensive
- C. Administer IV hypotonic fluids
- D. Administer IV empiric antibiotics

ANSWER: D

73. A 20-year-old male university student came to an emergency department with a severe headache and fever accompanied by seizures, and a stiff neck of 12 hours duration. Physical examination reveals 110 heart beats per minute, 24 breaths per minute, temperature of 39.2 degrees Celsius, and blood pressure of 110/78mm/Hg; and has petechiae, splinter hemorrhages, and pustules. The patient experienced flexion of the hips and knees in response to passive neck flexion and contraction of the hamstrings in response to knee extension while the hip is flexed. Emergency department work-up suggests bacterial meningitis.

What is the most appropriate priority intervention in this case?

- A. Administer chemoprophylaxis
- B. Administer empiric antibiotic
- C. Prepare for lumbar puncture
- D. Prepare the patient for a CT scan

ANSWER: B

74. A 45-year-old male patient presented to an emergency department with fever, nausea, vomiting, and generalized body weakness of three days duration. On examination, he has a descending symmetric paralysis, pupils are dilated and nonreactive to light, and there is a large wound on the left lower leg covered with bandage and cloths. Dysarthria and dysphasia are also noticed. The work-up suggests botulism poisoning.

What is the most appropriate priority nursing intervention in this case?

- A. Administer antibiotics
- B. Administer antitoxin
- C. Administer corticosteroids
- D. Administer anticholinergic

ANSWER: B

75. A 29-year-old male patient was referred from a health center with a diagnosis of Guillain-Barré syndrome. Vital signs show Blood pressure of 120/80mmHg, pulse rate of 78 beats per minute, respiratory rate of 14 breaths per minute, and oxygen saturation of 98 percent.

What is the most likely immediate complication in this case?

- A. Respiratory failure
- B. Dysrhythmias
- C. Septic shock
- D. Heart failure

ANSWER: A

76. A 29-year-old patient with traumatic brain injury was admitted to an intensive care unit. Vital signs show blood pressure of 70/60mmHg, pulse rate of 58 beats per minute, respiratory rate of 8 breaths per minute, temperature of 38.5 degrees centigrade, and oxygen saturation of 85 percent. The emergency nurse identified that intracranial pressure was not controlled, and herniation syndrome occurred due to brainstem compression.
What is the most likely complication?
A. Cardiorespiratory arrest
B. Cardiac dysrhythmias
C. Metabolic alkalosis
D. Metabolic acidosis
ANSWER: A
77. A 24-year-old male patient came to an emergency department with non-bloody diarrhea, fever, and abdominal pain of three days duration. At the time of presentation, vital signs reveal Blood pressure of 90/70 mmHg and Temperature of 38 degrees centigrade. The emergency nurse suspects the diagnosis of infectious gastroenteritis.
What is the most likely pertinent history that gives a clue to suspect the diagnosis in this case?
A. Age of the patient
B. Duration of the diarrhea
C. Non-bloody Diarrhea
D. Fever and abdominal pain
ANSWER: D
78. A 20-year-old male patient presented to an emergency department with right lower abdominal pain and vomiting of three hours duration. An emergency nurse tells the patient to be in a supine position on the examination couch, and then rotating the flexed right thigh at the hip internally and externally elicits pain in the right lower abdomen.
What most likely physical examination did the emergency nurse work out?
A. Psoas sign
B. Rovsings' sign
C. Cullen's sign
D. Obturator sign
ANSWER: D
79. A 39-year-old female patient is presented to an emergency room with a complaint of non-radiating epigastric pain of four days duration in which the pain is relieved with ingestion of food. Physical examination reveals epigastric area tenderness.
What is the most likely diagnosis in this case?
A. Upper gastrointestinal bleeding
B. Peptic Ulcer Disease
C. Pancreatitis
D. Peritonitis
ANSWER: B

80. A 37-year-old patient is presented to an emergency room with bloody vomiting and bright red rectal bleeding of three hours duration.
What is the most likely diagnostic investigation that would differentiate the type of gastrointestinal bleeding in this case?
- A. Platelet count
 - B. Endoscopy
 - C. Hematocrit value
 - D. Abdominal Ultrasound
- ANSWER: B
81. An emergency practitioner wants to confirm upper gastrointestinal bleeding by nasogastric aspirate. During the procedure, suddenly, blood clots are seen in the gastric aspirate.
What is the most appropriate nursing care plan in this case?
- A. Providing sips of water
 - B. Giving intravenous fluid
 - C. Performing Nasogastric feeding
 - D. Performing gentle gastric lavage
- ANSWER: D
82. A 27-year-old patient is presented to an emergency room with watery diarrhea of two hours duration. The patient has no nausea and vomiting. Vital signs show Blood pressure of 110/70mmHg, Respiratory rate of 16 breaths per minute.
What is the most appropriate nursing care plan in this case?
- A. To give Oral hydration with a glucose-based solution
 - B. To give Oral hydration with caffeine-enriched fluid
 - C. To administer Intravenous crystalloid Fluid
 - D. To administer Intravenous dextrose Fluid
- ANSWER: A
83. A referred adult patient is presented to an emergency unit with a complaint of bloody vomiting. Vital signs reveal Blood pressure of unrecordable, pulse rate of 130 beats per minute, and Respiratory rate of 26 breaths per minute.
What is the most appropriate initial management in this case?
- A. Intubation
 - B. IV fluid administration
 - C. Nasogastric feeding
 - D. reversal of coagulopathy
- ANSWER: B
84. An acutely sick-looking patient with crampy abdominal pain and vomiting of one day duration came to the hospital. Physical examination reveals high-pitched and tinkling with loud rushes, tachycardia, poor skin turgor, dry mucous membrane, visible peristalsis and distention, hyperactive bowel sound in the early course, and later decreased bowel sound. Finally, the health care provider suspects bowel obstruction.
What is the most likely definitive management in this case?
- A. Oral fluid administration
 - B. Antibiotic administration
 - C. Surgical intervention
 - D. Oxygen administration
- ANSWER: C

85. A 32-year-old female patient is presented to an emergency room with complaints of bloody diarrhea and general weakness of one day duration. Physical examination shows dry mouth, poor skin turgor, and altered mental status.
What is the most priority nursing intervention in this case?
- A. Airway clearance
 - B. Intravenous hydration
 - C. Positioning the patient
 - D. Counsel the patient to avoid caffeine
- ANSWER: B
86. A 33-year-old male patient presented to an emergency room with abdominal distension, severe crampy abdominal pain of five hours duration. After surgical consultation, the physician decided for surgery and the patient signed consent.
What is the next most appropriate nursing intervention to be done before surgery?
- A. Giving oral fluid without giving solid food
 - B. Intravenous broad-spectrum antibiotics
 - C. Cardiopulmonary resuscitation
 - D. Oxygen administration
- ANSWER: B
87. A physician decided to intubate a patient with a diagnosis of upper gastrointestinal bleeding before resuscitating the hemodynamically unstable patient.
For what condition is the patient at greatest risk?
- A. Peri-intubation hypotension
 - B. Clot formation from aggregation
 - C. Peri-intubation bleeding
 - D. Post-intubation bleeding
- ANSWER: A
88. A 42-year-old patient with a history of repeated upper gastrointestinal bleeding is presented to an emergency department having active bleeding of one hour duration. Vital signs show a heart rate of 130 beats per minute, Blood pressure of 70/50mmHg, and GCS of 8.
For what condition is the patient at greatest risk?
- A. Infection
 - B. Aspiration
 - C. cirrhosis
 - D. Bowel obstruction
- ANSWER: B
89. A 28-year-old acutely ill male patient is presented to an emergency department with chills, fever, and flank pain of one day duration. Objective data shows a temperature of 38 degrees centigrade, and Costovertebral angle tenderness. Laboratory investigation shows leukocytosis and bacteriuria. An emergency nurse suspected the diagnosis of acute pyelonephritis.
What is the most relevant clinical manifestation to diagnose acute pyelonephritis?
- A. Costovertebral Angle tenderness
 - B. Acutely ill looking
 - C. Leukocytosis
 - D. Flank pain
- ANSWER: A

90. A 67-year-old male patient who refused surgical removal of benign prostate hypertrophy came to an emergency department after two months with urinary retention of one day duration. On assessment, he has edema, crackles, shortness of breath, distended neck veins, and there is progressive, irreversible deterioration of renal function.
What is the most likely actual nursing diagnosis in this case?
- A. Imbalanced Nutrition
 - B. Excess fluid volume
 - C. Deficient fluid volume
 - D. Risk for infection
- ANSWER: B
91. A 59-year-old male patient is presented to an emergency department because of the inability to urinate for the last 12 hours. He had also had urine urgency and frequency. After further assessment, a physician diagnosed it as benign prostate hypertrophy.
What is the most appropriate initial management in this patient's care?
- A. Removal of benign prostate hypertrophy by surgery
 - B. Removal of urine using an indwelling bladder catheter
 - C. Giving oral antibiotics to decrease the risk of infection
 - D. Giving oral fluid to increase urination
- ANSWER: B
92. A 37-year-old known heart failure patient is presented to a hospital with shortness of breath for five hours duration. Physical examination reveals distended neck veins, crackles on auscultation of the lung surface, tachycardia, and increased blood pressure. Then, an emergency nurse put the patient on supplemental oxygen and prescribed medication.
What is the priority nursing intervention to monitor the progress?
- A. Measuring input and output
 - B. Giving sodium-enriched oral fluid and food
 - C. Turning and positioning the patient regularly
 - D. Educating the patient about the disease process
- ANSWER: A
93. A 34-year-old patient is presented to an emergency department with excessive vomiting and diarrhea of two days duration. The patient also complains of orthostatic lightheadedness and decreased urine output.
For what condition is the patient at greatest risk?
- A. Post renal kidney injury
 - B. Prerenal kidney injury
 - C. Intrarenal kidney injury
 - D. Immediate renal failure
- ANSWER: B
94. A 20-year-old patient who was treated for pharyngitis last week came to an emergency department with darkened urine, edema, fever, and general malaise of one day duration.
For what condition is the patient at greatest risk?
- A. Prerenal kidney injury
 - B. Intrarenal kidney injury
 - C. Post renal kidney injury
 - D. Pulmonary edema
- ANSWER: B

95. A 40-year-old known hyperthyroidism female patient came to an emergency department with palpitations, restlessness, and hyperactivity. On physical examination, *she* has excessive sweating, warm and moist skin, widened pulse pressure, tachycardia with atrial fibrillation, exophthalmia, and increased perspiration.

What is the most relevant history the nurse should anticipate for this diagnosis?

- A. Weight gain
- B. Heat intolerance
- C. Decreased appetite
- D. Excessive urination

ANSWER: B

96. A 19-year-old female patient came to an emergency department with complaints of restless movements and hyperactive reflexes. Her history suggests nervousness, insomnia, heat intolerance, and moderate weight loss despite an increased appetite. On physical examination, *she* has excessive sweating, palpitations, 144/98mmHg blood pressure, 134 heart beats per minute, 32 breaths per minute, and 38.2 degrees Celsius temperature.

What is the most likely diagnosis in this case?

- A. Hyperthyroidism
- B. Cushing's syndrome
- C. Diabetic ketoacidosis
- D. Adrenal insufficiency

ANSWER: A

97. A 57-year-old known diabetic female patient came to an emergency department with altered mental status. On physical examination, the patient has deep and rapid breathing, dehydration, lethargy, stupor, semi-consciousness, and has fruity odor during breathing. Laboratory results show a glucose level of 420mg/dL, urine ketone 4+, and a normal white blood cell count. An emergency nurse started facemask oxygen, cardiac monitor, and intravenous fluid resuscitation.

What is the next most appropriate nursing management plan for this patient?

- A. Check serum sodium level
- B. Administer insulin infusion
- C. Check serum potassium level
- D. Administer potassium chloride

ANSWER: C

98. A 55-year-old female patient came to an emergency department after experiencing severe chest pressure, nausea, and diaphoresis. *She* has not been diagnosed with diabetes before. On physical examination, the patient is fully oriented, obese, with slightly diaphoretic skin, dry buccal membranes, and has blood pressure of 166/116mm/Hg, heart beats of 112 per minute, respiration rate of 28 breaths per minute, and temperature of 37.6 degrees Celsius.

What is the most appropriate nursing management plan in this case?

- A. Check serum glucose level
- B. Maintain normal blood pressure
- C. Advise the patient on regular exercise
- D. Administer adequate intravenous fluid

ANSWER: A

99. A 25-year-old male patient is presented to an emergency department after experiencing confusion, lethargy, and deep and rapid breathing. On physical examination, he has fruity odor breathing, dry buccal membranes, blood pressure of 110/86 mm/Hg, heart beats of 96 per minute, respiration rate of 24 breaths per minute, temperatures of 36.8 degree Celsius, blood glucose level of 368 mg/dl, and urine ketone of 4+. The patient is stupor and semi-conscious. An emergency nurse started facemask oxygen, aggressive fluid resuscitation, and insulin infusion, bilateral with potassium chloride. What is the next most appropriate management that the nurse should anticipate for this patient?
- A. Treating the precipitating factors
 - B. Administer broad-spectrum antibiotics
 - C. Check serum glucose levels every 30 to 60 minutes
 - D. Check serum sodium level every 30 to 60 minutes

ANSWER: C

100. A 35-year-old female patient came to an emergency department with delirium, intermittent seizures, and coma. The patient has a history of heat intolerance, tremors, palpitations, hyperactivity, and excessive body sweating. Physical examination revealed tachycardia, excessive body temperature of 40 degrees Celsius, and a diagnosis of thyrotoxicosis. What is the most appropriate emergency nursing priority intervention for this patient?
- A. Treating the patients' psychological fear
 - B. Treating the precipitating factors
 - C. Administer adequate fluid resuscitation
 - D. Control excessive fluid overload

ANSWER: C

101. A 38-year-old female patient came to an emergency department with tremors, palpitations, hyperactivity, and excessive body sweating. *She* has a history of heat intolerance, tachycardia, and excessive body temperature of 39.6 degrees Celsius. *She* is diagnosed with thyrotoxicosis. An emergency nurse started cardiac monitoring and intravenous fluid resuscitation. What is the most expected complication management the nurse should anticipate in this case?
- A. Increased weight gain
 - B. Increased body fluid volume
 - C. Decreased serum glucose level
 - D. Disturbed serum electrolyte panels

ANSWER: D

102. A 50-year-old male patient came to an emergency department with fever, sweating, hyperactivity, and nervousness accompanied by delirium, seizures, and coma. He is a known type I diabetic patient on follow-up with sliding scale insulin therapy. On presentation, he has Polydipsia, polyuria, dehydration, Kussmaul's respirations, mental status changes, and 'fruity breathe'. His blood glucose level is 600 mg/dL, urine ketone is 3+, and PH is 7.07. What is the most expected common complication the nurse should anticipate in this case?
- A. Diabetic Ketoacidosis
 - B. Hyperglycemic Hyperosmolar Syndrome
 - C. Hypoglycemia
 - D. hyperglycemia

ANSWER: A

103. A 19-year-old female patient came to an emergency department with a history of headache, fever, and otalgia. On physical examination, an emergency nurse noted a bilateral swelling of the tonsils with an abscess. The emergency nurse suspected a Peri-tonsillar abscess.

What is the most relevant history that should be asked by the nurse, in addition?

- A. Dysphagia
- B. Vomiting
- C. Diarrhea
- D. Nausea

ANSWER: A

104. A person with a sudden onset of cough, difficulty speaking, and signs of difficulty in gas exchange came to the emergency room. On presentation, an emergency nurse observed that the person had grasped his neck with both hands, and he was agitated.

What is the most likely diagnosis in this case?

- A. Tonsillar-pharyngeal abscess
- B. Foreign body obstruction
- C. Tonsillo-pharyngeal mass
- D. Tracheo-laryngeal edema

ANSWER: B

105. A 29-year-old male patient came to an emergency department with fever, sore throat, otalgia, and odynophagia of five days duration. He was treated for a peritonsillar abscess two years before. Physical examination reveals inferior and medial displacement of the right-sided tonsil and palatal edema. A work-up suggests peritonsillar abscess, but the diagnosis is not confirmed.

What is the most appropriate care plan to confirm the diagnosis of this patient?

- A. Identify detailed history
- B. Conduct Intraoral US
- C. Send the patient for a CT scan
- D. Detailed physical examinations

ANSWER: B

106. A 20-year-old male patient came to an emergency department with nasal bleeding of about 30 minutes duration. An emergency nurse guided the patient to lean forward in the "sniffing" position and pinch the soft nares between the thumb and the middle finger for a full 15 minutes and breathe through the mouth. When evaluated after 15 minutes, the bleeding did not stop and continued.

What is the most appropriate next care plan for this patient?

- A. Rapid reduction of blood pressure
- B. Continue applying direct nasal pressure
- C. Reversal of coagulopathy with blood products
- D. Apply chemical cauterization with silver nitrate

ANSWER: B

107. A 25-year-old male football player came to an emergency department with bleeding from the nose. He says that he clashed with his friend while playing football. On physical examination, respiration rate of 20 breaths/minute, heart rate of 100 beats/minute, and blood pressure of 120/70 mmHg.

What is the most appropriate emergency management in this case?

- A. Apply tetracycline ointment pack on the left nostril
- B. Apply direct pressure on the soft palate and lining forward.
- C. Put the patient on the bed and apply packing to the nostril.
- D. Clean the blood from the patient and send for X-ray

ANSWER: B

108. A 20-year-old female patient came to an emergency department with complaints of sore throat, fever, headache, and fatigue for 12 12-day duration. *She* was not able to breathe properly. On assessment, febrile to touch and is diagnosed as a peri-tonsillar abscess.

What is the most likely complication in the management of this patient?

- A. Damage to the carotid arteries
- B. Dissemination of infection
- C. Aspirations of gastric contents
- D. Perforation of the oral cavity

ANSWER: A

109. A 27-year-old male patient with known HIV/AIDS experiences skin reaction at the site of injection, which is characterized by erythema, sclerosis, and swelling for two days duration. Previously, he had no history of allergy.

What most appropriate physical examination to be used to describe the skin reaction?

- A. Inspection
- B. Palpation
- C. Percussion
- D. Auscultation

ANSWER: A

110. A 28-year-old newly diagnosed HIV/AIDS male patient was admitted to an emergency department for blood transfusion. During transfusion, the patient is complaining of hemolytic transfusion reactions, difficulty breathing, redness, and itching on the skin. On laboratory investigation, both IgG and IgM increased.

What is the most likely hypersensitivity reaction that occurred in this patient?

- A. Type I (immediate type)
- B. Type II (tissue-specific)
- C. Type III (immune complex-mediated)
- D. Type IV (delayed type; cell-mediated)

ANSWER: B

111. A 35-year-old health care provider was exposed to a needle stick injury at the emergency department while he was giving an injection to a known HIV- positive patient. What is the most likely nursing intervention in this case?
- A. Cleaning the area with water and soap
 - B. Give post-exposure prophylaxis (PEP)
 - C. Give antiretroviral therapy (ART)
 - D. Give tetanus antitoxin (TAT)
- ANSWER: A
112. A 25-year-old female patient with known HIV/AIDS came to an emergency department with anaphylaxis to the prescribed antiretroviral medication regimen of Acyclovir, Famciclovir, and Valacyclovir. *She* developed a rash and itching. What is the priority nursing action in this case?
- A. Administer adrenaline
 - B. Administer antipyretic
 - C. Administer antibiotic
 - D. Administer anti-pain
- ANSWER: A
113. A 37-year-old newly diagnosed HIV/AIDS male patient received the prescribed antiretroviral treatment regimen of Acyclovir, Famciclovir, and Valacyclovir for a one-month duration according to a physician's order. What is the most likely purpose of this treatment regimen?
- A. Cure the HIV/AIDS infection
 - B. Advanced T-cell CD4⁺ count
 - C. Prevent opportunistic infection
 - D. Realize viral replication load
- ANSWER: D
114. A 45-year-old male patient with known HIV/AIDS came to an emergency department with a diagnosis of nephropathy. What is the most likely potential complication in this case?
- A. Renal insufficiency
 - B. Ischemic heart disease,
 - C. Congestive heart failure
 - D. Hypersensitivity reaction
- ANSWER: A
115. A 25-year-old female patient came to an emergency room after a motor bicycle accident with complaints of severe chest pain and slight bleeding from the lower part of her right leg. On physical examination, pulse rate is 110 beats per minute, blood pressure is 115/75mmHg, and respiratory rate is 23 breaths per minute. What is the most likely triage category of this patient?
- A. Yellow
 - B. Orange
 - C. Green
 - D. Red
- ANSWER: B

116. A 29-year-old male patient came to an emergency department after a road traffic accident with a history of chest pain, shortness of breath, two episodes of vomiting, and pain on inspiration. The emergency nurse suspects a rib fracture.

What is the most relevant history to suspect a rib fracture in this patient?

- A. Vomiting
- B. Bleeding
- C. Pain on inspiration
- D. Road traffic accident

ANSWER: C

117. A 36-year-old female patient came to an emergency department with a complaint of difficulty breathing after a road traffic accident. Vital signs show blood pressure of 130/65mmHg, pulse rate of 110 beats per minute, and respiratory rate of 29 breaths per minute. On further physical examination, there is tracheal deviation to the affected part and asymmetrical chest movement. An emergency nurse suspects pneumothorax.

What is the most likely percussion finding in this case?

- A. Dullness
- B. Tympanic
- C. Tactile fremitus
- D. Hyper resonance

ANSWER: D

118. A 35-year-old male patient came to an emergency department after a road traffic accident with complaints of severe pelvic pain and tenderness over the symphysis pubis. An emergency nurse suspects an unstable pelvic fracture.

What is the most relevant clinical manifestation to suspect this diagnosis?

- A. Ecchymosis on pubis
- B. Local edema of the pelvis
- C. Tingling of the pubis
- D. Open-book type pelvis

ANSWER: D

119. A 26-year-old male patient came to an emergency department with a head injury after a motor vehicle accident of two days duration. On physical examination, there is decreased consciousness, hemiparesis, and unilateral pupillary abnormalities. His vital signs show blood pressure of 145/95mmHg, Pulse rate of 58 beats per minute, and respiratory rate of 12 per minute.

What is the most likely diagnosis for this patient?

- A. Contusion injury
- B. Subdural hematoma
- C. Epidural hematoma
- D. Diffuse axonal injury

ANSWER: B

120. A 31-year-old female patient was referred to a tertiary hospital after one month of severe head injury, and *she* was admitted to the intensive care unit for better care. On physical examination, blood pressure is 125/70mmHg; pulse rate is 85 beats per minute, and respiratory rate is 19 breaths per minute. On further assessment, *she* opens her eyes spontaneously, and involuntary lip smacking, chewing, roving eye movements, and inability to respond to the environment were noticed.

What is the most likely diagnosis for this patient?

- A. Subarachnoid hemorrhage
- B. Vegetative state coma
- C. Diffuse axonal injury
- D. Epidural hematoma

ANSWER: B

121. A 41-year-old female patient came to an emergency department with complaints of severe headache and frequent episodes of vomiting after two days duration of head injury. An emergency nurse suspects increased intracranial pressure.

What is the appropriate care plan for this patient?

- A. Positioning the patient in the prone position
- B. Positioning the patient in the supine position
- C. Positioning the patient in the reverse Trendelenburg position
- D. Positioning the patient by elevating the head of the bed to 30 degrees

ANSWER: D

122. A 43-year-old male patient was admitted to an intensive care unit with severe brain injury. He was on a mechanical machine for respiratory support. An intensive care unit nurse plans care to control intracranial pressure.

What is the most appropriate care plan for this patient?

- A. Initiate the Valsalva maneuver
- B. Avoid excessive suctioning
- C. Maintain cerebral perfusion pressure at 58mm Hg.
- D. Administer O₂ to maintain PaO₂ at 60 mm Hg

ANSWER: B

123. A 25-year-old male patient came to an emergency department after a motor vehicle crash with midfacial fractures and swelling. Vital signs show blood pressure of 120/75mmHg, pulse rate of 110 beats per minute, and respiratory rate of 21 breaths per minute. Further physical examination reveals bilateral orbital ecchymosis and cerebrospinal fluid leak. An emergency physician diagnosed the case as a Le Fort injury.

What is the most appropriate initial care plan for this patient?

- A. Securing the IV line with fluid
- B. Providing IV antibiotics
- C. Protecting the airway
- D. Nasal packing

ANSWER: C

124. A 25-year-old male patient came to an emergency department with complaints of severe chest pain and anxiety after a car accident. Vital signs show a pulse rate of 110 beats per minute and a respiratory rate of 25 breaths per minute. Further assessment showed there is an open chest wound and hyperresonance on the percussion of the chest.

What is the most appropriate initial care plan for this patient?

- A. To allow the flow of air to the chest
- B. To insert a chest tube for blood removal
- C. To facilitate postural drainage
- D. To insert a needle decompression

ANSWER: D

125. A 39-year-old female patient came to an emergency department after a road traffic accident. On physical examination, a nurse noted bleeding from the nose, bruising over the mastoid, cerebrospinal fluid mixed with blood from the ears, and a GCS of 8. Her vital signs show blood pressure of 90/65mmHg, pulse rate of 110 beats per minute, respiratory rate of 28 breaths per minute, and oxygen saturation of 85 percent.

What is the most appropriate initial management for this patient?

- A. Providing oral fluid
- B. Inserting an NGT tube for feeding
- C. Performing endotracheal intubation
- D. Providing oxygen via a nasal catheter

ANSWER: C

126. A 45-year-old male patient came to an emergency department after a road traffic accident of six hours duration. On physical examination, blood pressure of 110/65mmHg, pulse rate of 110 beats per minute, respiratory rate of 30 breaths per minute, and oxygen saturation of 78 percent were found. He opens his eye painfully and speaks inappropriate words; and extension of his extremities on pressure stimuli was noticed.

What is the most appropriate management for this patient?

- A. Aggressive fluid resuscitation
- B. Endotracheal intubation
- C. Oxygen via nasal cannula
- D. Request for MRI

ANSWER: B

127. A 46-year-old man was brought to an emergency department after falling accident of approximately one week ago. He does not recall the event, and his family stated that he is sleeping more than usual and seems forgetful. An emergency physician sent for a brain CT scan, and the result is normal except for the injury to the head.

What is the most appropriate management for this patient?

- A. Admit the patient to the intensive care unit
- B. Educate his family for reassurance
- C. Provide anti-pain medications
- D. Put a patient on oxygen

ANSWER: B

128. A 45-year-old female patient came to an emergency department after a road traffic accident with complaints of acute pain in the lower extremities. Physical examination reveals edema, deformity, crepitus, and ecchymosis on the left leg. Vital signs show blood pressure of 110/65mmHg, pulse rate of 110 beats per minute, respiratory rate of 20 breaths per minute, and Oxygen saturation of 94 percent.

What is the most appropriate emergency management for this patient?

- A. Hold the affected part in an elevated position
- B. Educate for active exercise immediately
- C. Immobilize the affected part
- D. Hot therapy of the affected part

ANSWER: D

129. A 29-year-old male patient was referred to an emergency department with the diagnosis of a closed right lower leg fracture. After assessment, the emergency team reduced, immobilized the fracture, and edema was controlled. Emergency nurses monitored circulation, motion, and sensation routinely. After a week, the patients' condition had improved.

What is the next most appropriate nursing care management for this patient?

- A. Isometric and muscle-setting exercises
- B. Encourage the patient to take oral fluid intake
- C. Encourage patient coughing exercise
- D. Educate the accident prevention

ANSWER: A

130. A 25-year-old male patient came to an emergency department after a motor vehicle crash with a fracture of the knee cap of two days duration. He is presented with acute pain, cannot ambulate, and is notably edematous due to fluid.

What is the most likely management for this patient?

- A. Perform arthrocentesis
- B. Elevate the affected leg
- C. Apply a splint to the kneecap
- D. Provide antibiotics to the knee cap

ANSWER: A

131. A 20-year-old female patient developed burns following exposure to hot water while *she* was cooking. The burn covers the anterior trunk and both of her forearms. Her body weight is 60kg (use Parkland formula).

What is the most appropriate amount of fluid required over 24 hours?

- A. 6900ml
- B. 6480ml.
- C. 6100ml
- D. 5900ml

ANSWER: B

132. A 35-year-old male patient working at a steel factory was exposed to Nitric acid. The acid was poured on his left leg, and he came to an emergency department within five minutes of exposure. On physical examination, the affected leg became yellowish. His vital signs showed a heart rate of 105 beats/minute, blood pressure of 100/70mmHg, and oxygen saturation of 96 percent at room air.

What is the most appropriate management for this patient?

- A. Transfer to the burn unit
- B. Open IV line
- C. Apply colloid
- D. Hydrotherapy

ANSWER: D

133. A 30-year-old female patient working in a factory experienced circumferential partial thickness flame burn to her chest and neck. *She* was referred to the burn center four days after the incident. On physical examination, *she* has decreased air entry and shallow breathing. *She* is conscious and able to communicate.

What is the most appropriate management in this case?

- A. Escharotomy
- B. Fasciotomy
- C. Cover wound
- D. Apply cream

ANSWER: A

134. A 34-year-old male patient developed a second-degree burn following exposure to superheated oil. The wound covers the anterior torso area, both arms, and the genitals. He complains of severe pain. His vital signs showed blood pressure of 135/80mmHg, heart rate of 105 beats/minute, and oxygen saturation of 96 percent at room air.

What is the most appropriate pain management for this patient?

- A. IV morphine
- B. PO morphine
- C. PO acetaminophen
- D. Ibuprofen

ANSWER: A

135. A 70-year-old patient fell on the flame of firewood. Immediately, a bystander called an emergency medical technician to the scene. His clothes were burning when an Emergency medical technician arrived at the scene, and the safety of the scene was secured.

What is the most appropriate next step to manage this patient?

- A. Assess the airway
- B. Assess breathing
- C. Patient extrication
- D. Remove burning cloth

ANSWER: C

136. A 33-year-old male patient was exposed to white phosphorus while preparing insecticide. He came to an emergency department with complaints of burns to his hand, leg, and upper torso. He claims that the droplets of powder of white phosphorus dropped on his clothing. Also, he is claiming direct contact with the powder.

What is the most appropriate care for this client?

- A. Remove his clothing and brush it away
- B. Remove his clothing and send it to the laundry
- C. Remove his clothes and irrigate with water
- D. Use copper sulfate to detoxify

ANSWER: C

137. A 23-year-old female patient sustained pelvic injury while *she* was crossing a road. *She* was admitted to the orthopedic ward, and *she* has been on morphine 5mg IV every four hours. *She* was complaining of breakthrough pain, and *she* took orally prepared morphine. After two hours of ingestion, *she* develops CNS manifestations as evidenced by miosis. Her vital signs showed blood pressure of 100/60mmHg, respiration rate of 11 beats/minute, and oxygen saturation of 95 percent at room air.

What is the most appropriate treatment in this case?

- A. Atropine
- B. Calcium
- C. Magnesium sulphate
- D. Naloxone

ANSWER: D

138. A 33-year-old male patient came to the toxicology emergency after intentional ingestion of a large amount of acetaminophen. He does not remember the actual dose of the drug. He has nausea, vomiting, and right upper quadrant abdominal tenderness. He came after six hours of ingestion.

What is the most appropriate treatment?

- A. Activated charcoal
- B. Gastric lavage
- C. N-Acetylcysteine
- D. Offer milk

ANSWER: C

139. A 42-year-old female patient came to an emergency room after unintentional ingestion of bleach. *She* came with complaints of gastritis and nausea. *She* appears restless, and her vital signs show blood pressure of 128/70mmHg, heart rate of 105 breaths/minute, oxygen saturation of 95 percent of room air, and temperature of 36.7 degrees centigrade.

What is the most appropriate management in this case?

- A. Gastric lavage
- B. Offer water
- C. Induced vomiting
- D. Multiple-Dose Activated Charcoal

ANSWER: B

140. A 31-year-old male patient was brought to an emergency room after ingestion of 35 percent potassium hydroxide. On physical examination, drooling, edema, and hoarseness are noted. On vital signs, blood pressure of 70/45mmHg, heart rate 125beats/ minute, and respiratory rate of 11 breaths/minute and oxygen saturation of 55 percent at room temperature were noted. What is the most appropriate initial management in this case?

- A. Intranasal oxygen
- B. Oral intubation
- C. Insert airway
- D. Fluid management

ANSWER: B

141. A 38-year-old male patient came to an emergency department after ingestion of supra supra-therapeutic level of non-steroidal anti-inflammatory drug. He appears restless. He developed seizures and respiratory distress. Airway protected, and he is on full support of a mechanical ventilator.

What is the next most appropriate management in this case?

- A. Hemodialysis
- B. Urine alkalization
- C. Activated charcoal
- D. Exchange dialysis

ANSWER: C

142. A 55-year-old patient came to emergency room after unintentional exposure to chlorinated hydrocarbon insecticides, so-called DDT. He appears combative, having muscle twitching and an acute onset of seizure activity. He has a history of skin contact with the chemical.

What is the most appropriate initial management for this patient?

- A. Decontamination with normal saline
- B. Decontaminate with povidone iodine
- C. Decontaminate with 75 percent alcohol
- D. Decontamination with soap and water

ANSWER: D

143. A 35-year-old male patient sustained a head injury following a motor motorcycle accident. On initial presentation to an emergency room, his Glasgow coma scale (GCS) is 14, and vital signs show that blood pressure of 130/70mmHg, pulse rate of 95 beats/minute, and respiratory rate of 20 breaths/minute. After six hours of admission, the patient was re-evaluated, and the GCS became 11.

What could be the most likely clinical severity in this case?

- A. Moderate traumatic brain injury
- B. Mild traumatic brain injury
- C. Severe traumatic brain injury
- D. Mild to severe traumatic brain injury

ANSWER: A

144. A 40-year-old male patient was brought to an emergency room after he fell from a six-meter-high tree. He sustained a hyperextension cervical injury and femoral bone fracture. Cervical collar applied at the scene. Airway protected by an endotracheal tube of 7, 22cm at the lip. On physical examination, the patient has warm and dry skin. His vital signs showed blood pressure of 80/40mmHg, heart rate of 55 beats/minute, and oxygen saturation of 96 percent on 40 percent of FiO₂.

What is the most likely complication in this case?

- A. Spinal shock
- B. Neurogenic shock
- C. Hypovolemic shock
- D. Septic shock

ANSWER: B

145. A 60-year-old male patient came to an emergency room with a complaint of severe chest pain. In history, he sustained a blunt thoracic injury. A widened mediastinum was noted on a supine chest radiograph. His vital signs showed a heart rate of 119 beats/minute, respiratory rate of 25 breaths/minute, and oxygen saturation of 80 percent at room air. On physical examination, a pulse deficiency was noted in the arm.

What is the most likely complication in this case?

- A. Aortic injury
- B. Pneumothorax
- C. Tension Pneumothorax
- D. Cardiac tamponade

ANSWER: A

146. A 28-year-old male patient was admitted to the orthopedic ward with a right tibial bone fracture. Currently, he has developed pain, pallor, pulselessness, paresthesia, and paralysis on the affected leg. He complains of pain on passive stretching of the right leg. On examination with Stryker, the pressure is 35mmHg.

What is the most likely complication?

- A. Reflex Sympathetic Dystrophy
- B. Compartment syndrome
- C. Necrotizing fasciitis
- D. Avascular Necrosis

ANSWER: B

147. A 26-year-old woman came to an emergency department with complaints of severe abdominal pain, heavy and irregular vaginal bleeding for the last one month. On the physical examination, her vital signs show 90/60mmHg of blood pressure, 66 beats per minute of pulse rate, 18 breaths per minute of respiration rate, and 9mg/dl of hemoglobin.

What could be the most likely cause in this case?

- A. Early pregnancy
- B. Multiple preterm deliveries
- C. Antepartum hemorrhage
- D. Abnormal uterine bleeding

ANSWER: D

148. A 27-year-old pregnant mother came to gynecology ward with Abnormal Vaginal Bleeding (AUB). The physical examination showed 80/50mmHg of blood pressure, 68 beats per minute of pulse rate, 16 breaths per minute of respiration rate, and 8mg/dl of hemoglobin record.

What is the priority nursing intervention in this case?

- A. Administer Fluid and blood resuscitation
- B. Put the patient on a face mask oxygen
- C. Put the patient in the supine position
- D. Identify and treat underlying causes

ANSWER: A

149. A one-month-old neonate was admitted to a neonatal intensive care unit (NICU) with severe dehydration. On physical examination, sunken eyes, poor skin turgor, and decreased urine output were observed by a neonatal intensive care unit nurse.

What is the most relevant history used to diagnose severe dehydration?

- A. Mode of delivery
- B. Time of admission
- C. Family history of allergy
- D. Maternal breast-feeding habit

ANSWER: D

150. A one-year-old infant was admitted to a neonatal intensive care unit with suspected acute meningitis. On physical examination, febrile seizure, neck stiffness, petechial rash, and bulging fontanel were recorded. Before starting the management, an emergency and critical care nurse wanted to confirm the diagnosis of the admitted infant.

What most relevant investigation that should be conducted to confirm the diagnosis?

- A. Blood sodium
- B. Blood glucose
- C. Lumbar puncture
- D. Complete blood count

ANSWER: C

151. A. A 6-year-old child was brought to a pediatric emergency department by his mother with complaints of vomiting, diarrhea, and unable to drink. On physical examination, unconsciousness, sunken and skin return after pinching greater than two seconds were documented.

What is the most likely diagnosis in this case?

- A. Dehydration
- B. Mild dehydration
- C. Severe dehydration
- D. Moderate dehydration

ANSWER: C

152. A 5-year-old child was admitted to an emergency pediatric department with the diagnosis of severe dehydration. On physical examination, the nursing assessment indicates the rapid onset of severe dehydration symptoms, and the child is unable to maintain consciousness.

What is the most likely definitive treatment for this child?

- A. Intravenous fluid administration
- B. Administering antibiotics
- C. Oxygen administration
- D. Blood transfusion

ANSWER: A

153. A 57-year-old male patient was admitted to an emergency department with a diagnosis of acute psychosis. On assessment, physical examination shows he has a history of substance use. His vital signs revealed 100/60mmHg of blood pressure, 76 beats per minute of pulse rate, 16 breaths per minute of respiration rate, and 93 percent of oxygen saturation.

What is the most relevant clinical manifestation used to diagnose this case?

- A. Hallucination
- B. Delusion
- C. Delirium
- D. Dementia

ANSWER: A

154. A 37-year-old female patient was admitted to a psychiatric department after a suicidal attempt following a drug overdose. On the assessment, physical examination revealed *she* has a history of depression and recently interpersonal conflicts.

What is the most appropriate management for this patient?

- A. Administer stress management techniques
- B. Initiating one-on-one observation
- C. Initiating a group on one observation
- D. Administering an antidepressant drug

ANSWER: B

155. An emergency nurse who is working at a zonal health office as a quality officer asked the *Woreda* health office to submit a measles report. After he received the report, he checked the report's completeness and adherence to the recommended national reporting format.

What surveillance type is most likely this?

- A. Active surveillance
- B. Passive surveillance
- C. Sentinel site surveillance
- D. Laboratory surveillance

ANSWER: B

156. In a district, there were internally displaced people living in the camp. The World Food Program (WFP) wants to assess the nutritional status of under-five children, pregnant and lactating mothers by using a simple, cost-effective, and convenient way.

What most appropriate nutritional assessment method to be used in this case?

- A. Clinical method
- B. Dietary History
- C. Anthropometric
- D. Biochemical

ANSWER: C

157. A team from the World Food Program (WFP) assessed all vulnerable children, pregnant and lactating women of a district for malnutrition problems during an emergency time. They are provided with ready-to-use supplementary feeding.

What screening type is most likely used in the above scenario?

- A. mass screening
- B. multiphase screening
- C. case finding screening
- D. targeted screening

ANSWER: A

158. A Zonal Public Health Emergency Management (PHEM) team had two days review meeting with a *Woreda* PHEM focal person. The meeting focused on assessing performance on the disease surveillance system. The meeting ended after revising the diseases to be under surveillance using criteria.

What is the most likely criterion the team could use in this case?

- A. Fast recovery
- B. High prevalence
- C. Low incidence
- D. Incurable

ANSWER: B

159. The Gender and HIV AIDS directorate of a university conducts a training needs assessment for university students. The needs assessment result shows that boys usually have a fear of visiting health facilities due to cultural barriers and peer influence. The directorate gave training on the importance of early treatment of STIs for university students.

What disease prevention method level is most likely used by the directorate in this case?

- A. Prevention of exposure
- B. Primary prevention
- C. Secondary prevention
- D. Tertiary prevention

ANSWER: C

160. In a town, there is an epidemic of measles. The town health office planned to create awareness among the families about the prevention, treatment, and effects of the diseases. What is the most appropriate method of teaching to be used by the health office in this case?

- A. Counseling
- B. Home Visits
- C. Mass media
- D. Group Education

ANSWER: C

161. A six-year-old male child is admitted to a hospital after having a left-hand fracture. The child is crying continuously due to the severe pain that does not respond to the lower-level analgesics. A nurse who was in charge of a ward uses her hand clamp and tries to play with the child most of her time while *she* is in the ward. The child gets calm and stops crying due to the nurse's action.

What most appropriate palliative care component did the nurse use in this case?

- A. Non opioid analgesia
- B. Opioid analgesia
- C. Pharmacological
- D. Non-pharmacological

ANSWER: D

162. A 26-year-old university student came to an emergency OPD after *she* sustained a dog bite with complaints of pain and swelling over the right lower leg lasting for three days. On arrival, the wound is covered with a piece of bandage, and it oozes fluid.

What most appropriate first aid care is suitable for this victim?

- A. Clean with antiseptic solution, Suture and dress the wound
- B. Advice to observe the unusual behavior of the dog for 10 days
- C. Thoroughly clean the wound with soap and water
- D. Administer TT injection as per the physician's order

ANSWER: C

163. In December 2019, the novel coronavirus emerged in China. The Ethiopian Ministry of Health feared that it would become a pandemic and a public health problem. The Ministry of Health prepared an isolation area, restricted out and in migration, and applied strict screening of people at the border.

What disaster management phase is most likely explained in the above case?

- A. Acute phase
- B. Pre-emergency phase
- C. Impact and flight phase
- D. Post emergency phase

ANSWER: B

164. A high number of girls became impregnated after a war took place in most areas of a community. The number of STI infections, including HIV, was observed from a hospital's report.

What is the most likely public health impact of the disaster that explains this case?

- A. Sexual violence
- B. Psychological violence
- C. Emotional violence
- D. Physical violence

ANSWER: A

165. A government planned to enroll a public servant in social health insurance to prevent catastrophic health expenditure. The government enrolled health professionals in a pilot study. The health professionals who were enrolled in the social health insurance started to preach to other civil servants to be beneficiaries of social health insurance.

What is the most likely disaster management measure that explains this case?

- A. Escape routes
- B. Communications
- C. Utility shut-off and safety
- D. Insurance and vital records

ANSWER: B

166. A district health office warns the community to utilize insecticide-treated bed nets (ITN), suspecting the occurrence of a malaria epidemic due to climate change and an extended rainy season.

What type of emergency response phase is most likely addressed in this case?

- A. Pre-emergency phase
- B. Rehabilitation phase
- C. Acute phase
- D. Planning phase

ANSWER: A

167. The head of an emergency department plans to conduct a measles campaign by identifying responsible individuals to be involved in the campaign, when to do the campaign, and the number of days required for the campaign.

What type of plan is most likely developed by the head?

- A. Tactical
- B. Strategic
- C. Operational
- D. Contingency

ANSWER: C

168. A huge number of casualties from a car accident came to an emergency department at night, creating an unusual situation. The department head is the only responsible person to manage this situation by providing clear directions and by making the entire focus of the department on managing this situation.

What most appropriate leadership style is applicable in this case?

- A. Autocratic
- B. Democratic
- C. Laissez-faire
- D. Charismatic

ANSWER: A

169. A newly appointed hospital manager transfers a nurse who serves as the director of nursing in an emergency department to another unit. The nurse then refuses the transfer and takes his complaint to the hospital's administration. The hospital's administration accepts his complaint and has decided to return him to the previous position.

What conflict resolution strategy most likely explains this situation?

- A. Win-win
- B. Lose-lose
- C. Win-lose
- D. Lose-win

ANSWER: C

170. A head of a maternal and child health department in a primary hospital performs monthly supervision and guides the staff on how to develop objectives and an action plan, prepare a schedule, make registrations, and attend labor.

What supervisory role is most likely applied in this case?

- A. Mentor
- B. Coach
- C. Advocate for the organization
- D. Advocate for the employee

ANSWER: B

171. A Non-Governmental Organization has implemented a project on accident management and prevention in a Zone for three years, and currently phased out. The Zonal health office wants to conduct an evaluation of the project regarding awareness, attitude, and behaviors related to accident prevention and management in the Zonal community.

What evaluation type could be applied in this situation?

- A. Process evaluation
- B. Impact evaluation
- C. Summative evaluation
- D. Input evaluation

ANSWER: B

172. A 25-year-old female client came to use the voluntary counseling and testing service. *She* is screened for HIV, and her result is positive. A service provider counseled and informed her about her result. *She* accepts her result and asks the provider to keep her result secret. A health care provider explains that it is his responsibility to keep this type of confidential issue.

What is the most likely risk category explained in this case?

- A. Medical staff-related risk
- B. Patient care-related risk
- C. Employee-related risk
- D. Financial risk

ANSWER: B

173. A head of a surgical ward wants to look at how often vital signs are taken during the first ten hours after surgery. He explains that post-operative vital signs are to be checked every 30 minutes for two hours and thereafter every hour for eight hours.

What is the most likely service quality audit process explained in this case?

- A. Make a judgment about quality
- B. Collect and analyze information
- C. Establish standard/ criteria
- D. Compare collected information with the standard

ANSWER: C

174. A director of HIV/AIDS control in Ethiopia wants to measure the quality of HIV care services in hospitals. Then, he assessed the proportion of hospitals with medical doctors trained on HIV care, the proportion of hospitals with ART drugs, and the availability of laboratory services for HIV testing to use it as a quality measure dimension.

What is the most likely dimension of quality measure assessed in this situation?

- A. Outcome
- B. Structure
- C. Process
- D. Impact

ANSWER: B

175. A medical doctor who is working as a director of the TB clinic assessed service provision to TB patients. He identifies that the patients who are on Direct Observation Therapy (DOT) did not take Isonazid Prophylactic Therapy (IPT), and contact tracers were not done for all TB patients. Finally, he decides to give training for health care professionals who are working on the TB clinic.

What is the most appropriate reason to conduct a service quality audit in this case?

- A. To educate participants
- B. To improve patient care
- C. To increase job satisfaction
- D. To criticize a health care professional

ANSWER: B

176. A head of medical ward assessed the quality of service using the proportion of nurses who use the nursing process to provide nursing care as one indicator. Based on his assessment, almost all nurses do not use the nursing process while giving care to the patients. Therefore, the head arranged training on the nursing process for the medical ward staff. Finally, he planned to monitor the medical ward at least once a week.

What is the most likely quality audit stage applied in this case?

- A. Setting criteria
- B. Planning to audit
- C. Comparing performance
- D. Sustaining improvements

ANSWER: D

177. An emergency and critical care nursing researcher is interested in seeing the effect of physiotherapy on the recovery rate of patients with stroke. The researcher enrolled 120 stroke patients and grouped them into two in which one group received physiotherapy, but the other did not.

What is the most likely study design to be used in this case scenario?

- A. Prospective cohort
- B. Case control
- C. Experimental
- D. Retrospective cohort

ANSWER: C

178. Researchers conducted a study to identify factors associated with the high prevalence of mental health problems in a community. During the data collection period, they promised the community that they would report the findings of the study. But they didn't report the true findings to the community.

What is the most likely ethical rule the research team breached in this study?

- A. Veracity
- B. Privacy
- C. Confidentiality
- D. Fidelity

ANSWER: A

179. An emergency and critical care nurse student is interested in determining a newborn's body temperature immediately after birth. The student measured the temperature of the newborn using a thermometer on a Fahrenheit scale.

What is the most likely scale of measurement to be used in this case scenario?

- A. Nominal scale
- B. Ordinal scale
- C. Interval scale
- D. Ratio scale

ANSWER: D

180. A researcher conducted a case-control study to identify determinants of stroke 10 years ago. The researcher selected those individuals who recovered from a stroke, and another group of people without a stroke. Then, the researcher asked about their previous exposure to the risk factors of stroke.

What is the most likely bias that could occur in this study?

- A. Observer bias
- B. Recall bias
- C. Interviewer bias
- D. Detection bias

ANSWER: B

181. Graduates of emergency and critical care nursing students were assigned to one of the health centers in their nearby University. Their supervisors instructed them to do research on "Hand washing practice following toilet use among adult population in the community". They selected data collectors and assigned them to the catchment *kebeles* to collect data.

What is the most likely data collection method to be used in this case scenario?

- A. Face-to-face interview
- B. Direct observation
- C. Self-administered questionnaire
- D. Telephone interview

ANSWER: B

182. A researcher is interested in conducting a study on "Assessment of breast self-examination practice among female preparatory school students". The researcher has a separate list of female students from the office of the school director. The researcher then decided to select a representative sample of female students.

What is the most likely sampling technique to be used in this case scenario?

- A. Simple
- B. Systematic
- C. Stratified
- D. Cluster

ANSWER: A

183. A researcher wants to determine the birth weight of newborns delivered at a hospital. The researcher took a sample of 340 newborns and took their weight from the registration book. The researcher took the following 8 samples of birth weight in kg from the hospital's delivery registration book: 2.4, 3.8, 2, 3, 4, 2.5, 3, 4.5.

What is the median weight of the children in kg in this scenario?

- A. 2.5
- B. 3.0
- C. 3.15
- D. 3.5

ANSWER: B

184. A diarrheal disease was reported in a *kebele* among under-five children. The *kebele* data shows that out of the 250 under-five children living in the *kebele*, 25 of them contracted diarrheal disease.

What is the cumulative incidence of diarrhea in this report?

- A. 5
- B. 10
- C. 50
- D. 65

ANSWER: B

185. An instructor is going to deliver a course for his masters' students. The instructor is interested in using the best teaching aid which allows students to pay better attention, take fewer notes, and helps them to emphasize on main points.

What is the most appropriate teaching aid to be used in this case?

- A. Books
- B. CD-ROOMs
- C. Handouts
- D. Flip charts

ANSWER: C

186. An emergency care and critical care nursing instructor discussed with facilitators about the visual aid to use in the training. The instructor wants to display prepared notes, training objectives, and summarize the experiences after clinical practices.

What is the most appropriate visual aid to be used in this case scenario?

- A. Flipcharts
- B. Writing board
- C. Slides
- D. Video

ANSWER: A

187. On the first day of a class, an instructor provided a course syllabus for the students and discussed the contents with them to give an overview of the course, and directed the students.

What is the most likely correct statement that indicates the course syllabus in this case?

- A. Describe what graduates must do after completing the competencies
- B. Indicate what students should do after completion of a chapter of the course
- C. Explain what students should do after completion of the course
- D. Describe the general course contents and objectives

ANSWER: D

188. An emergency and critical care nursing instructor is assigned to provide a first aid course for the students. The instructor prepared a lesson plan about cardiac and respiratory arrest. The instructor presented the steps that are necessary to perform CPR for victims with both conditions.

What is the most appropriate teaching method to be used in the above case scenario?

- A. Clinical simulation
- B. Case study
- C. Interactive presentation
- D. Demonstration

ANSWER: D

189. A nurse working in an intensive care unit washes her hands before and after each procedure to prevent the spread of diseases and infections from one patient to another.

What is the most appropriate ethical principle applied in this case?

- A. Justice
- B. Autonomy
- C. Beneficence
- D. Nonmaleficence

ANSWER: C

190. A nurse was preparing a patient for a surgical procedure. The patient didn't know about his current condition and a complication that would happen after the procedure.

What is the most likely ethical issue related to the patients' rights?

- A. Right to truth
- B. Right to refuse treatment
- C. Right to give informed consent
- D. Right to participate in human experimentation

ANSWER: A

191. A nurse is caring for a patient admitted to an intensive care unit with a diagnosis of respiratory failure. The patient was intubated, and he was on a mechanical ventilator. The nurse actively connected with the patient's emotions by putting herself in the patient's position. The nurse applied one of the compassionate care elements to the patient.

What is the most likely relational communication applied in this case?

- A. Affect
- B. Demeanor
- C. Behaviors
- D. Engagement

ANSWER: A

192. A medical student was assigned to an emergency room to take history and perform a physical examination for bedside discussion. While performing a physical examination on the genital area, the student exposed the patient's privacy publicly. Thus, a nurse in charge there asked the student to use a screen to keep the patient's privacy.

What is the most likely nursing code of ethics applied in this case?

- A. Advocacy
- B. Accountability
- C. Responsibility
- D. Confidentiality

ANSWER: A

193. An emergency nurse is caring for a patient admitted to an intensive care unit. A physician reviewed the patient's chart to understand the current status of the patient and to decide what had to be done to facilitate his recovery. Based on the information recorded on the chart, the physician ordered some drug changes and recommended continual care.

What is the most likely purpose of documentation in this scenario?

- A. Education
- B. Reimbursement
- C. Communication
- D. Legal documentation

ANSWER: C

194. A nurse is assessing a patient who arrived at emergency room with a complaint of shortness of breath. A nurse assessed the patient's vital signs, recorded the observed and measured data, and also documented "The patient's pulse rate is 110 beats/min."

What documentation quality feature is most likely applied by the nurse in this case?

- A. Factual
- B. Accurate
- C. Complete
- D. Organized

ANSWER: A

195. An instructor nurse supervises students at an emergency ward. The instructor gave an assignment to take the history of at least one patient in an emergency room and to report their findings. During evaluation, one student reflected that the patient has consumed an adequate amount of water. But the instructor advised the student to document the intake as 500 ml of water.

What is the most likely characteristic of quality documentation suggested by the instructor?

- A. Timeliness
- B. Accurateness
- C. Completeness
- D. Organization

ANSWER: B

196. A nurse working in an emergency room recorded a patient's information in an organized way using common vocabulary terms that allow the nurses to provide continuity of care. The handwriting is legible, but there is an erased and overwritten word for medication errors. Abbreviations that are acceptable to colleagues and easily understandable are used.

What is the most likely activity that makes the nurse legally safe?

- A. Erasing medication errors
- B. Recording the data in an organized way
- C. Using abbreviations known to his colleague
- D. Legible handwriting and overwritten words on erased words

ANSWER: B

197. A nurse working in an emergency room of the health center was assigned to provide health education in the early morning before work time. The nurse educated the community about typhoid fever, which is identified as the top priority problem of the community by the health center.

What is the most likely communication level that could be used by the nurse?

- A. Public communication
- B. Intrapersonal communication
- C. Interpersonal communication
- D. Transpersonal communication

ANSWER: A

198. A 50-year-old male patient was admitted to an emergency ward with a diagnosis of diabetes ketoacidosis. A nurse was educating the patient about the risk factors and the early signs and symptoms of diabetic Ketoacidosis. Finally, the nurse posed a question to the patient: "Tell me what you understood about the disease".

What is the most likely element of therapeutic communication applied in this case?

- A. Honesty
- B. Empathy
- C. Validation
- D. Trustworthiness

ANSWER: C

199. A 45-year-old male patient came to an emergency room with a complaint of bladder distension of 2 days duration. On assessment, a physician diagnosed urethral stricture, and a supra pubic catheter should be inserted as a temporary measure. However, the patient refused to give consent.

What is the most likely action to be taken by the nurse?

- A. The nurse should coerce the patient to give consent
- B. The nurse should seek consent from the family member
- C. The patient's refusal must be documented in the medical record
- D. Since the procedure is urgent, the patient should sign after the procedure

ANSWER: C

200. A nurse working in an emergency room of a health center has a good relationship with local leaders living in the catchment area to provide health education. The nurse uses newspapers for those who can read, radio, and television to share information and discuss important issues with the population.

What is the most likely form of professional nursing relationship applied in this case?

- A. Nurse family relationship
- B. Nurse community relationship
- C. Nurse-patient helping relationship
- D. Nurse health care team relationship

ANSWER: B

